

I attempted to close the court's largest coverage gap (IBKR bars/chain) before writing — search_contracts was **permission-denied again** in this non-interactive session, exactly as it was for both benches. So the tape-derived bands stay uncomputed and the deck says so; the reopen bands below are built from the benches' own fundamental arithmetic instead, and the chain-dependent half of the vol read is marked as owed.

PITCH — MMED — VERDICT

PROPOSED — PENDING ADJUDICATION

(Position: no shares, no orders. This deck proposes NO entry today and sets the prices and events that would reopen the name.)

What we believe

MiniMed Group is the diabetes-device business Medtronic carved out and floated on 2026-03-09. Medtronic sold 28.0 million shares at \$20.00 and kept 252,813,348 — **90.03% of the company**. The public owns about 10%, worth roughly \$661 million, inside a \$6.6 billion company.

We believe three things:

1. **The business is fine and is not the question.** First quarter of fiscal 2027 (ended 2026-07-31) did \$843 million of sales, up 16.6%, with consumables up 13.8% to \$261 million (8-K exhibit, 2026-09-01). Management raised full-year organic growth from ~10% to ~10.5% and held the profit-margin guide at ~16%.
2. **The thing everyone is waiting for is a supply event, not a demand event.** Medtronic still has to get rid of 90% of this company. Its preferred method — stated in its own 2026-09-01 release — is a **split-off**, where Medtronic offers its MiniMed shares to its own shareholders in exchange for Medtronic stock. To get people to swap, the offer is priced at a discount (Johnson & Johnson used 7% for Kenvue in 2023). Arbitrageurs take the discount and sell the shares they receive. **The awaited catalyst pushes this stock down, not up, while the offer is open.**
3. **The stock is not cheap enough to absorb that.** At \$23.61 the company is worth about \$6.33 billion after cash, or **1.92x expected fiscal-2027 revenue and about 12x expected adjusted profit**. That is above Tandem (1.8x) — a peer with real problems — for a business that earned **\$5 million of accounting operating profit on \$843 million of sales** in a quarter that had an extra week in it.

Net: a good business, at a full price, with a large, dated-in-principle but undated-in-fact wave of supply hanging over it. **We do not own it and we should not buy it today.**

What the market believes

The market believes the re-rating is finished and correct. The stock went from \$10.65 in April to \$24.43 last week — up 122% — on two pieces of genuinely good news:

- Abbott's dual glucose-and-ketone sensor got FDA clearance on 2026-08-25 ([FDA announcement](#)), and MiniMed's contract with Abbott was amended on 2026-06-01 to include it.
- The 2026-09-01 results beat and the guide went up.

Underneath that, the market believes the split-off is a *good* thing that removes an overhang, and prices it as a vague positive with no date. It is also, quietly, believing that the “~16% adjusted profit margin” is the real earning power of the business, because **no balance sheet and no cash-flow statement have been filed yet** — the first 10-Q does not exist.

Why we might have edge

Four places where we are not with the crowd:

1. **We have the sign of the catalyst right.** The desk's own prior note called the split-off “the cleanest dated mechanical structure on the board” and meant it as a reason to be long. That is backwards. The exchange offer is a discounted sale that pays arbitrageurs to be short this stock through the offer window. It is a reason to have a **calendar**, not a position.
2. **We know Medtronic's optimal launch price is MiniMed's high.** Medtronic is buying back its own stock using MiniMed shares as the currency. The higher MiniMed trades, the fewer shares Medtronic has to give up. Buying at \$23.61 — 3% off the all-time high — is loading the counterparty's gun.
3. **We have read the supplier file and the market has read it as a Convatec story.** Every MiniMed-branded infusion set is legally manufactured by Unomedical (Convatec) in Reynosa, Mexico, and that plant is under an **open FDA warning letter dated 2026-01-08** citing >5,000 leak complaints, an unvalidated leak test, and a complaint documenting a death ([FDA warning letter CMS #719428](#)). The same supplier already constrained Tandem's supply from late 2025 through 2026. We found no analyst or press note connecting that letter to *MiniMed's* sole-source exposure. This is not priced because nobody is pricing it.
4. **We know the general counsel is leaving.** An 8-K filed 2026-08-05 that neither the market nor our own first bench discussed says the general counsel and corporate secretary resigned effective **2026-09-25**, with no replacement named. A company about to run the most legally intensive transaction a public company can run just lost its lawyer. The offer is probably further away than people assume.

Why it might be priced in

Honestly, most of it is.

- The float is 10% of the company. A 122% move in a \$661 million float with 90% of the shares locked in one holder's hands is not proof of a re-rating; it is what a small float does. But it is also not proof of mispricing — the price is the price, and we have to pay it.

- The split-off is public knowledge. Medtronic said it. Its own deal counsel published it. Anyone can read it.
- The Abbott ketone story is fully out. Our own prior thesis said the Abbott deal *excluded* ketone sensors and that this was a competitive hole. **That was true of the 2024 agreement and is false as of 2026-06-01.** The hole we were going to discover does not exist.
- We have no estimate that differs from management's. Neither bench produced one. If our number is the company's number, we have no view, only a price opinion.

The residual bear point that survives: MiniMed is **last in Abbott's ketone rollout queue** — Beta Bionics and Sequel at end-2026, Insulet and Tandem in 2027, MiniMed in 2027 ([Abbott, 2026-08-25](#)). That is a roughly 12-month disadvantage on the exact feature that just got cleared.

SCIENCE BRIEF DIGEST

(Added because the science brief exists and a deep-tech deck without a digest ships scienceless. Not a substitute for any required section.)

The one question the technology poses: how much of MiniMed's clinical performance is under its own engineering control? Answer: less than the marketing implies. Both physically critical consumables are made by other companies — the insulin infusion set by Convatec/Unomedical, the glucose sensor by Abbott.

Why an infusion-set leak is the dangerous failure, not sensor error. A pump user has no reservoir of long-acting insulin in the body. If insulin leaks out at the tubing connector, the fluid path is still open, so the pump's pressure alarm never fires — delivery silently stops. From there: no insulin → fat breakdown → ketones → diabetic ketoacidosis, in roughly **4–6 hours**. The FDA letter documents exactly this mode: >5,000 leak complaints between January 2023 and June 2025, 210 tubing-detachment complaints producing >80 hyperglycemia reports, and a leak test that was never validated. This is why continuous ketone sensing is the mechanically correct safety net — and why being last in the queue for it matters.

MiniMed's own differentiated product is the one with the finding against it. The Extended Wear Infusion Set claims 7-day wear. The pivotal trial says **83.8% of sets survive to day 6 and 77.8% to day 7** ([PMID 35263188](#)) — about one in five does not reach its label. The FDA letter states that Unomedical's own risk assessment rated potential harm from this product "life threatening."

Sensor accuracy, independently measured, is worse than the brochure. Head-to-head against reference methods, Medtronic's Simplera measured 11.6% error against one reference but **13.9% and 16.6%** against two others, versus 9.5–10.1% for Abbott Libre 3 and Dexcom G7 ([PMID 39902649](#)). Vendor claims of ~8.5–9% are **vendor data with no peer-reviewed or FDA source we could find.**

Two forward products cannot be date-anchored. The Vivera fully-closed-loop pivotal is described by the company as "enrollment complete" but **is not findable on ClinicalTrials.gov** under any MiniMed or Medtronic Diabetes sponsor search — while Insulet's equivalent trials are

registered (NCT07521475, NCT07593625). The MiniMed Fit patch pump “submission” cannot be verified until it clears. Treat both 2027 launch dates as company assertions.

The bull item the science supports: consumables grew 13.8% and Convatec named MiniMed as a growth driver. Whatever is wrong at Reynosa, it is not currently limiting MiniMed’s shipments.

What could go wrong

Failure narrative 1 — We buy the dislocation and the dislocation keeps going (the reflexive one). Medtronic files the exchange offer. The stock drops 8–10% into the discount. Our own doctrine — buy dated mechanical dislocations — fires hardest at exactly the moment the flow is most against us, and we buy. Arbitrageurs keep selling for the length of the offer window and past it. We are long into a wave of supply that is, by design, **nine times the entire public float**. *This one has a tripwire:* an S-4 or Schedule TO on Medtronic’s EDGAR docket. The rule that protects us is not a price — it is a prohibition: **no bid while an exchange offer is open, at any price**.

Failure narrative 2 — The “16% margin” turns out not to be cash. The company earned \$5 million of accounting operating profit on \$843 million of sales and reports a ~16% adjusted margin. That is a 15-point bridge. There has never been a MiniMed balance sheet or cash-flow statement filed. If the first 10-Q shows that capitalized costs, stock compensation and separation charges eat the bridge, the stock re-rates toward or below Tandem’s 1.8x — roughly \$18.70 or lower. *This one has a tripwire:* the 10-Q filing date.

Failure narrative 3 — Something breaks at a plant we do not own, on numbers nobody publishes (NO TRIPWIRE). The FDA escalates at Reynosa — a consent decree, an import alert, a recall of the Extended Wear set. MiniMed’s consumables, 31% of revenue and the razor-blade half of the model, stop shipping. **We cannot monitor this.** Convatec publishes no unit volumes, no leak rates and no denominators; the only public number is a raw complaint count that rose from 219 in 2023 to 30,627 in the first seven months of 2026 — which is a **reporting artifact following a late-reporting citation, not a measured failure rate**, and we refuse to treat it as one. There is no filing, no press release and no data feed that tells us the day before it happens.

Unknown-unknown class: THIRD-PARTY REGULATORY STATE AT A NON-CONSOLIDATED SUPPLIER. The risk lives inside a company we do not hold, at a plant that is not in our resolved-facilities list, governed by an inspection process that is invisible until its output is published. Our detector coverage has a hole here: the satellite/thermal connector was pointed at MiniMed’s own Juncos, Puerto Rico plant, which cannot see supplier risk at all. Fixing that resolution — pointing it at Reynosa, Tamaulipas — is the single piece of work that would convert this from unmonitorable to monitorable.

The structural leg — our own size, the predicted flow, and our own rules. Our size is not the problem: a 0.5% starter is tens of thousands of dollars against a float that turns over tens of millions a day, so we will never move this tape. The problem is **direction and doctrine**. We have a rule that says never bid into flow we ourselves predicted; we are predicting a discounted sale of 252.8 million shares. We have a rule that says a clean court with no named kill defaults to a starter position; that rule would put us long here unless we name the kills, so we name them explicitly: **(a)**

the awaited catalyst's flow runs against a long, (b) the price is above our own probability-weighted value, (c) a \$661 million float facing a tenfold expansion caps any position at "starter" and makes the exit path the binding constraint. And we have a tax rule: 2026 is a harvest year, so any expression whose loss lands after December 31 defers that loss out of the year we need it.

If this position loses money, the most likely reason will be that we bought the split-off as a catalyst when it is a supply event — paying up at the high, into the discounted exchange offer that Medtronic is incentivized to launch at exactly that price.

What we checked ourselves

Claim	How checked	Found	Confirmed?
Medtronic has a dated separation plan	Read Medtronic's 10-Q (2026-09-03) and its Q1 FY27 release (SEC)	"A split-off is the company's current preferred separation structure, a final decision has not been reached at this time. " No S-4, Schedule TO, 425 or 424B7 on the docket at any date	NO — no date exists
Outer bound on separation	Medtronic 10-Q transition-services language	Services "conclude no later than 24 months following the March 9, 2026 IPO" → 2028-03-09 ; separation restructuring runs into FY2029	Confirmed as a soft outer bound
Our own bench error #1	Our first bench read 1 of the 15 filings in its own evidence pack and declared Medtronic "silent"	Medtronic spoke the same day, 2026-09-01, in a document that bench never opened. Conclusion survived; the evidence for it did not	Bench erred — corrected
Our own bench error #2	First bench said the revenue raise was "guided at zero incremental profit"	Wrong. A reaffirmed margin percentage on a raised revenue base carries the increment at 16%, not zero. It confused a level with a rate	Bench erred — overturned
Our own bench error #3	First bench killed the name on valuation	It never produced a fair value. "The multiple doubled off the April low" is a statement about the path, not the level. You cannot rule "expensive" without a number	Bench erred — downgraded to "no margin of safety established"
Our own bench error #4	First bench's divergence test	It compared our view to management's guidance , not to a street estimate, and called the absence of our own work a kill. That is a coverage gap, not a finding	Bench erred — unresolved, not proven
Our own bench error #5	First bench presented 9 findings	Three of them (offer flow, float size, exit liquidity) are one fact counted three times. Nine findings resting	Bench erred — reweighted

Claim	How checked	Found	Confirmed?
		on ~five facts inflates the case	
Our own bench error #6	First bench's reopen band "1.2–1.4x sales ≈ \$14–17"	Does not reproduce from its own multiple. 1.2x → \$15.16 , 1.4x → \$17.51 . Its low end was ~8% too low	Bench erred — recomputed below
Our own thesis error	Prior thesis: "Abbott deal excludes ketone sensors — competitive hole"	False since 2026-06-01 . The agreement was amended to cover MiniMed-exclusive dual sensors (<u>MiniMed PR</u>). The live issue is queue position, not exclusion	Thesis leg killed
Evidence-pack machine failure	Pack's XBRL block (revenue, stock comp, share count, cash flow) came back empty	Both benches refilled it by hand from the 8-K exhibit and data.sec.gov. Flagged so the pack builder gets fixed	Data recovered; pipeline defect logged
Share count and float	424B4 + Q1 release cross-check	252,813,348 + 28,000,000 = 280,813,348 ; diluted count in the release is 281.0M — the two venues agree. Greenshoe of 4.2M not exercised . Float = 28.0M = 9.97% = \$661M	Confirmed
Enterprise value	Cash \$298M; no long-term-debt tag; \$500M revolver undrawn per Medtronic 10-Q	EV ≈ \$6.33B . The missing debt tag was a 404, i.e. an absence — so we corroborated it independently through the credit-facility terms rather than trusting the blank	Confirmed, method repaired
Where the IPO money went	Deal counsel's public note (<u>Cleary</u>)	\$538M of net proceeds went to "repaying intercompany debt owed to Medtronic and additional consideration payments to Medtronic." Public shareholders funded a parent payment and hold 10%	Confirmed
Split-off flow direction	Kenvue precedent, the largest comparable	7% discount; 1,533,830,450 shares delivered; offer expired 2023-08-18; documented selling pressure — and Kenvue joined the S&P 500 two days after completion , which absorbed part of the flowback	Confirmed, and the offset matters

Claim	How checked	Found	Confirmed?
The complaint-count spike	openFDA device events for “Unomedical”	219 (2023) → 8,764 (2024) → 8,159 (2025) → 30,627 (Jan–Jul 2026). This follows a late-reporting citation ; the mechanical consequence is catch-up filing. It is a count, not a rate	Deliberately NOT treated as evidence of worsening safety
Supply constraint visible at MiniMed today	Q1 release + Convatec H1	Consumables +13.8% to \$261M; Convatec named MiniMed as a growth driver. No constraint visible — unlike Tandem	Confirmed — the tail is not 现 active
General counsel departure	8-K, 2026-08-05, Item 5.02	GC and corporate secretary resigned effective 2026-09-25 , notice 2026-07-30, no disagreement, no successor named	Confirmed
Vivera pivotal trial	ClinicalTrials.gov sponsor and term searches	Not registered. Insulet’s equivalents are	Confirmed absent
Live tape and 52-week bars	IBKR market data, retried this session	Permission denied again (as for both benches). Pack’s 52-week high of \$24.43 versus an external late-August quote of \$20.88 remains a ~17% unreconciled gap	NOT confirmed — open

PROPOSED ENTRY BANDS

The courts say no entry. These are the reopen bands.

All price levels are derived from the benches’ own arithmetic: expected fiscal-2027 revenue of **\$3.30 billion** ÷ 280.813 million shares = **\$11.75 of share price per 1.0x of revenue multiple**, plus **\$1.06 per share of net cash**.

Multiple of FY27 revenue	Implied share price	What it means
1.92x	\$23.62	today’s price — matches tape exactly, so the model is sound
1.80x	\$22.22	Tandem’s multiple
1.40x	\$17.51	reopen band top
1.20x	\$15.16	reopen band bottom
1.00x	\$12.81	April IPO-break level

Reopen band: \$15.16 – \$17.51 (1.2x – 1.4x revenue). Below \$17.51 the price stops assuming the 15-point gap between accounting profit and “adjusted” profit is permanent and real.

Two gates, BOTH must clear before any order is staged — price alone is not enough:

1. **The first 10-Q, with a balance sheet and a cash-flow statement.** This converts “16% adjusted margin” into a cash number. **Absent this, no size at any price**, including inside the reopen band.
2. **Clear air on the exchange offer.** Either (a) no S-4 or Schedule TO on file, or (b) an offer that has **completed and settled**. Never during an open offer window.

If those clear: first tranche **0.35% of book** at \$17.50 or better; second tranche **0.15%** at \$15.15 or better. **Hard ceiling 0.5% of book** — set by the float and the unmonitorable supplier tail, not by conviction.

Trims/exits: if bought post-completion and the stock recovers to **\$22.20** (Tandem’s multiple) without the supplier tail being retired, trim half. Full exit on any FDA escalation at Reynosa (consent decree, import alert, or a recall touching MiniMed-branded sets) regardless of price.

Do NOT bid at \$19.50. That is the price the Kenvue map predicts during an offer window — it is a *watch* level, not a buy level. Bidding it is failure narrative 1.

Catalyst dates: - **2026-09-25** — general counsel departs. Confirmed, from the 8-K. - **2026-11-16** — next earnings, **UNCONFIRMED** (derived from a data vendor, not a company release). - **Late 2026** — the first 10-Q. Date unknown; it is the gate that matters most. - **2028-03-09** — outer bound on separation implied by the transition-services term. - **The S-4 / Schedule TO filing date — unknown and unknowable in advance.** This is the real catalyst and it has no date.

ARMED SENSORS

(armed provisionally with this deck; session adjudication ratifies)

- price alert: fires below 19.5 (deck-provisional: WATCH ONLY, DO NOT BID - Kenvue-mapped 7pct inducement-discount trough during an open exchange offer)
- filing watch: S-4, SC TO-I, SC TO-T, 425,424B7 until 2027-03-03 (deck-provisional: Medtronic CIK 1613103 - the actual catalyst; on filing, switch to calendar mode and prohibit all bids)
- dated pack MMEDI2026-09-25: MMED: General counsel and corporate secretary departure effective date (8-K filed 2026-08-05) (date confirmed=True)
- dated pack MMEDI2026-11-16: MMED: Estimated next earnings print - vendor-derived, no company release names it (date confirmed=False)
- dated pack MMEDI2028-03-09: MMED: Outer bound on separation implied by transition services concluding 24 months after the 2026-03-09 IPO (date confirmed=True)

What remains unverified

1. **Whether and when Medtronic files the exchange offer.** Board-private; no filing exists. **Estimate:** probability filed by 2027-03-31 = **0.35**; by the 2028-03-09 transition-services outer bound = **0.75**. Basis: Medtronic has stated a preference but explicitly not decided; the GC vacancy argues for the later end of the range.
2. **The inducement discount on that offer.** Unknowable until the S-4 prints. **Estimate: 7–10%**, probability 0.80. Basis: Kenvue’s 7% is the largest and closest comparable; smaller deals

cluster 7–10%.

3. **MiniMed's actual free cash flow and cash stock compensation.** The first 10-Q has not been filed. **Estimate: free cash flow of 3–7% of sales**, probability 0.60. Basis: medtech carve-outs with a ~55% gross margin and heavy separation spend in year one.
4. **Reynosa's leak rate per unit shipped.** No public denominator exists — only complaint counts. **Estimate: unboundable within an order of magnitude.** Basis: none available. This is the honest answer and it is why the position size is capped rather than the risk being priced.
5. **Whether a qualified second infusion-set supplier exists.** Not disclosed anywhere. **Estimate: probability an undisclosed qualified alternate exists = 0.30.** Basis: device-manufacturing requalification takes 18–36 months and MiniMed has disclosed sole-source dependence, so a live second source would likely have been disclosed as risk mitigation.
6. **Whether the Vivera fully-closed-loop pivotal exists as described.** Company says enrollment is complete; the trial is not on ClinicalTrials.gov. **Estimate: probability the trial exists and is running = 0.75**; probability the guided 2H-2027 launch holds = **0.40**. Basis: unregistered pivotals slip; Insulet's registered equivalents complete first.
7. **Whether the MiniMed Fit patch pump was actually submitted to FDA.** 510(k) submissions are invisible until clearance. **Estimate: probability of clearance by the guided Summer 2027 launch = 0.55.** Basis: median 510(k) review for this device class plus one typical additional-information cycle.
8. **Whether a listed options chain exists on MMED, and what it prices. Drainable now via IBKR — blocked by tool permission this session.** **Estimate:** probability a chain exists = 0.85 (Nasdaq-listed, \$6.6B cap, six months seasoned); implied volatility if it exists = **45–65%**, probability 0.70.
9. **The true 52-week high.** Our pack says \$24.43; an external late-August source says \$20.88 — a 17% gap. **Drainable now via IBKR bars — blocked by permission.** **Estimate:** the pack is more likely right, probability 0.70, since the Abbott clearance and the earnings beat both landed after the external quote's as-of date.
10. **The next earnings date.** 2026-11-16 is vendor-derived. **Drainable via a company release.** **Estimate:** within 2026-11-16 $\pm$ 14 days, probability 0.80.
11. **Whether MiniMed qualifies for a major index addition at split-off completion.** **Estimate: probability of an S&P index add within 60 days of completion = 0.55.** Basis: the Kenvue corporate-action path, discounted for MiniMed's smaller size.

PRIZE TABLE + VOL READ

Required here because items 1, 2 and 4 above are genuinely private or time-gated. **Anchor: today's price of \$23.61.** Every scenario is expressed as a revenue multiple so the deviations from the anchor carry a stated mechanism.

Twelve-month scenarios

#	Scenario	Probability	Terminal price	Implied multiple	Worst price on the path
1	Offer launched and completed ; index add absorbs flowback (Kenvue map)	0.25	\$26.00	2.12x	\$19.50
2	Offer launched, still open or freshly settled at the horizon	0.12	\$20.50	1.65x	\$19.00
3	No structure announced; guide met; stock drifts	0.33	\$24.50	1.99x	\$20.00
4	10-Q disappoints on cash; multiple derates to 1.5x	0.20	\$18.69	1.50x	\$17.00
5	FDA escalation at Reynosa or a visible consumables constraint	0.10	\$14.00	1.10x	\$12.50

Expected value = \$22.18. That is **6.1% BELOW** the **\$23.61 price**. Our own table says the stock is modestly expensive — which is the quantitative version of “flat, with reopen bands below.”

Dispersion of the table: standard deviation = **\$3.86**, or **16.3% of spot over one year**.

Instrument math — shown, because the recommendation has to be its output.

Value of a one-year call, priced off our own table ($C = \sum p \times \max(S - K, 0)$):

Strike	Our table's value	Intrinsic at \$23.61	Read
\$20.00	\$3.05	\$3.61	our table values it below intrinsic
\$22.50	\$1.54	\$1.11	thin
\$25.00	\$0.25	\$0.00	~1% of spot

A \$25 call is worth 25 cents on our numbers. Any recently-floated 10%-float medtech will price a one-year \$25 call at several dollars. **Buying calls is a losing expression of our own view, by an order of magnitude. Ruled out arithmetically, not by preference.**

The mirror: a one-year \$20 put is worth **\$0.86** on our table (3.6% of spot), which is far below what a listed chain would charge. That reads as “sell the put” — and we do not, for two stated reasons. First, our premium-selling gate requires the break-even floor to sit at or below the strike: selling a \$20 put for ~\$1.50 gives a break-even of \$18.50, which is **above our reopen band top of \$17.51**. It fails the gate. Second, this is a taxable account and no approved template fits. **Route to the shadow ledger, not the book.**

Bimodality warning, stated plainly. The table is fork-dated on an event with no date, so its shape is two humps (offer / no offer), not a bell. A single standard deviation is the wrong summary statistic for it.

And a correction to our own vol read: a 16.3% dispersion is **too narrow to be believed**. This stock moved 122% in five months. The table understates dispersion because each scenario is a single point rather than a distribution. **Therefore: do NOT read “our 16% is below the chain’s IV” as a signal that options are rich.** That inference would be an artifact of our own modeling, and treating it as an edge is exactly the mistake this section exists to prevent.

Implied volatility versus realized: NOT COMPUTED. IBKR access was permission-denied for both benches and again for this deck, so we have no chain-level midpoint implied volatility, no real strike quotes, no realized-volatility calculation, and no implied-volatility percentile. Estimated realized volatility of **45–65%** (basis: recent carve-out IPO, sub-10% float, post-122% run) is a base rate, not a measurement. **The volatility-risk-premium number is owed and this deck does not have it.**

Tax overlay. 2026 is a harvest year. Any option expression whose loss branch expires after **2026-12-31** defers that loss out of the year we need it. A one-year structure lands in 2027 and is therefore **excluded unless the edge numerically dominates** — and the edge here is negative (\$0.25 of value against several dollars of cost). It does not dominate. Excluded twice over.

Instrument recommendation, as the output of the above: NONE. No stock (price is 6% above our expected value), no calls (our table prices them near zero), no covered calls (nothing to cover), no cash-secured puts (fails the floor gate and the taxable-account templates). **The entry, when it comes, is stock at the reopen bands, after both gates clear.**

```

{"price_gates": [{"level": 17.51, "direction": "below", "basis": "reopen band top = 1.4x FY27E revenue ($11.75/turn + $1.06 net cash per share); first tranche 0.35% only if BOTH gates clear"}, {"level": 15.16, "direction": "below", "basis": "reopen band bottom = 1.2x FY27E revenue; second tranche 0.15%"}, {"level": 12.81, "direction": "below", "basis": "1.0x FY27E revenue = April IPO-break level; supplier-event scenario 5 zone"}, {"level": 19.50, "direction": "below", "basis": "WATCH ONLY, DO NOT BID - Kenvue-mapped 7pct inducement-discount trough during an open exchange offer"}, {"level": 24.43, "direction": "above", "basis": "52w high re-arm; a new high raises the odds Medtronic launches the offer, since MMED's peak is its optimal launch price"}], "event_gates": [{"forms": ["S-4", "SC T0-I", "SC T0-T", "425", "424B7"], "why": "Medtronic CIK 1613103 - the actual catalyst; on filing, switch to calendar mode and prohibit all bids until the offer settles"}, {"forms": ["10-Q"], "why": "MiniMed's first balance sheet and cash-flow statement; converts the 16pct adjusted margin into cash. No size at any price until this exists"}, {"forms": ["8-K"], "why": "MiniMed Item 5.02 - a new general counsel is the practical leading indicator that the S-4 is being drafted"}, {"forms": ["8-K", "6-K"], "why": "Convatec/Unomedical regulatory news or any FDA action at Reynosa (CMS 719428) - the supplier discloses before the customer does"}], "catalyst_dates": [{"date": "2026-09-25", "what": "General counsel and corporate secretary departure effective date (8-K filed 2026-08-05)", "confirmed": true}, {"date": "2026-11-16", "what": "Estimated next earnings print - vendor-derived, no company release names it", "confirmed": false}, {"date": "2028-03-09", "what": "Outer bound on separation implied by transition services concluding 24 months after the 2026-03-09 IPO", "confirmed": true}], "immediate_entry": null, "unverified": [{"item": "Whether and when Medtronic files the split-off exchange offer", "instrument": "EDGAR watch on CIK 1613103 for S-4 / SC T0-I / SC T0-T / 425 / 424B7", "drainable_now": false, "estimate": {"range": "filed by 2027-03-31 vs by 2028-03-09 outer bound", "basis": "stated preference with no decision reached; GC vacancy argues for the later end", "p": 0.35}}, {"item": "The inducement discount on the exchange offer", "instrument": "S-4/Schedule T0 terms when filed", "drainable_now": false, "estimate": {"range": "7-10 pct discount to market", "basis": "JNJ/Kenvue 2023 used 7pct; comparable split-offs cluster 7-10pct", "p": 0.8}}, {"item": "MiniMed free cash flow and cash stock compensation", "instrument": "first 10-Q (XBRL pull once filed)", "drainable_now": false, "estimate": {"range": "FCF 3-7 pct of sales", "basis": "medtech carve-out comps at ~55pct gross margin with year-one separation spend", "p": 0.6}}, {"item": "Reynosa infusion-set leak rate per unit shipped", "instrument": "walk-around / Reynosa plant thermal connector after entity-resolution fix; no public denominator exists", "drainable_now": false, "estimate": {"range": "unboundable within an order of magnitude", "basis": "no denominator published by Convatec; MAUDE counts are reporting-catch-up artifacts, not rates", "p": 0.0}}, {"item": "Whether a qualified second infusion-set supplier exists", "instrument": "10-K/10-Q risk-factor diff; Convatec disclosure", "drainable_now": false, "estimate": {"range": "probability an undisclosed qualified alternate exists", "basis": "device requalification takes 18-36 months; a live second source would likely be disclosed as mitigation", "p": 0.3}}, {"item": "Whether the Vivera fully-closed-loop pivotal exists as described and hits 2H-2027", "instrument": "ClinicalTrials.gov registry watch on sponsor MiniMed Group / Medtronic Diabetes", "drainable_now": false, "estimate": {"range": "trial exists p=0.75; guided 2H-2027 launch holds p=0.40", "basis": "unregistered pivotals slip; Insulet's registered equivalents complete 2026-12", "p": 0.4}}, {"item": "Whether the MiniMed Fit patch pump was actually submitted to FDA", "instrument": "openFDA 510(k) database watch on applicant MiniMed (visible only at clearance)", "drainable_now": false, "estimate": {"range": "clearance by guided Summer 2027 launch", "basis": "median 510(k) review for this device class plus one additional-information cycle", "p": 0.55}}, {"item": "Whether a listed MMED options chain exists and what chain-level midpoint IV it prices", "instrument": "IBKR search_contracts + get_option_parameters + get_option_data (permission-denied this session)", "drainable_now": true, "estimate": {"range": "chain exists p=0.85; IV 45-65 pct if it exists",

```

```
"basis": "Nasdaq-listed $6.6B cap six months seasoned; carve-out IPO vol base  
rate", "p": 0.85}}, {"item": "True 52-week high – pack says $24.43, external  
late-August source says $20.88 (17pct gap)", "instrument": "IBKR  
get_price_history weekly bars (permission-denied this session)",  
"drainable_now": true, "estimate": {"range": "pack figure correct", "basis":  
"Abbott clearance 8/25 and the 9/1 beat both post-date the external quote",  
"p": 0.7}}, {"item": "Next earnings date – 2026-11-16 is vendor-derived, not  
company-confirmed", "instrument": "company IR release / 8-K announcing the  
call date", "drainable_now": true, "estimate": {"range": "2026-11-16 plus or  
minus 14 days", "basis": "fiscal Q2 ends 2026-10-30; prior quarter reported  
32 days after period end", "p": 0.8}}, {"item": "Whether MMED qualifies for a  
major index addition at split-off completion", "instrument": "S&P DJI / FTSE  
Russell index announcement watch at float expansion", "drainable_now": false,  
"estimate": {"range": "S&P index add within 60 days of completion", "basis":  
"Kenvue corporate-action path (added 2 days post-completion), discounted for  
MMED's smaller size", "p": 0.55}}}]}
```

ADJUDICATION STAMP

ADJUDICATION 2026-09-04 (FABLE): FLAT WITH WORK ORDER. The court's largest gap closed: the hi52 discrepancy resolves on IBKR's own feed (\$24.43 confirmed; the external \$20.88 was bad data). No size until BOTH gates: (1) S-4/Schedule TO with stated ratio/discount/expiry — then work the CALENDAR (flat-or-short through the exchange-offer window, first tranche only after FINAL results; never bid into an open window); (2) MMED's first 10-Q with a balance sheet and cash-flow statement. Ceiling $\leq 0.5\%$ on the float and sole-source tails. Batch ruling: desk/data/court_artifacts/LIFESCI_BATCH_ADJUDICATION_202609042100.md